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Question 1

Not answered

Marked out of 5.00

Physical signs/symptoms of intoxication

Five young men attend a substance misuse clinic with the following presentations. For each of the physical signs/symptoms of intoxication, select the most likely option from the list provided

Tachycardia, red eyes, lack coordination, postural hypotension

Choose... ▾

Sedation, dizziness, respiratory depression

Choose... ▾

Tachycardia, hypertension, anorexia, tremor and restlessness

Choose... ▾

Tachycardia, tachypnea, hypertension, headache, chest pain and respiratory depression

Choose... ▾

Pinpoint pupils, clammy skin, respiratory depression

Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Cannabis intoxication- Tachycardia, red eyes, lack of coordination, postural hypotension.
- 2) Benzodiazepines intoxication- Sedation, dizziness, respiratory depression.
- 3) Amphetamine intoxication- Tachycardia, hypertension, anorexia, tremor and restlessness.
- 4) Cocaine intoxication- Tachycardia, tachypnea, hypertension, headache, chest pain and respiratory depression.
- 5) Heroin intoxication- Pinpoint pupils, clammy skin, respiratory depression

(Ref: Maudsley prescribing guidelines- Pg 328)

The correct answer is: Tachycardia, red eyes, lack coordination, postural hypotension → Cannabis intoxication, Sedation, dizziness, respiratory depression → Benzodiazepines intoxication, Tachycardia, hypertension, anorexia, tremor and restlessness → Amphetamine

intoxication, Tachycardia, tachypnea, hypertension, headache, chest pain and respiratory depression → Cocaine intoxication, Pinpoint pupils, clammy skin, respiratory depression → Heroin intoxication



Question 2

Not answered

Marked out of 5.00

Mental state changes

Five young men attend the A& E department following ingestion of a recreational drug, with the following presentation. Select the most likely drug responsible for the mental state changes described below

Relaxation, dizziness and disorientation

Choose... ▾

Euphoria, paranoia, anxiety, panic attacks, insomnia

Choose... ▾

Drowsiness, euphoria, hallucinations

Choose... ▾

Auditory hallucinations, paranoia, decreased concentration

Choose... ▾

Elation, perceptual distortions, disturbance of memory/judgment

Choose... ▾

Your answer is incorrect.

Explanation: The drugs of misuse and most common mental state changes are described below.

1) Benzodiazepines can cause initial relaxation followed by dizziness and disorientation. Though barbiturates can do the same, the latter are not as commonly abused as the former.

2) Cocaine- Euphoria, paranoid psychosis, anxiety, panic attacks, insomnia and excitement.

3) Heroin (a type of opioid)- Drowsiness, euphoria, hallucinations.

4) Amphetamines- Auditory hallucinations, paranoia, decreased the concentration.

5) Cannabis- Elation, perceptual distortions, disturbance of memory/judgement. (Ref: Maudsley prescribing guidelines- Pg 328)

The correct answer is: Relaxation, dizziness and disorientation → Benzodiazepines, Euphoria, paranoia, anxiety, panic attacks, insomnia → Cocaine, Drowsiness, euphoria, hallucinations → Heroin, Auditory hallucinations, paranoia, decreased concentration → Amphetamine, Elation, perceptual distortions, disturbance of memory/judgment → Cannabis



Question 3

Not answered

Marked out of 5.00

Withdrawal symptoms

Five young men attend the substance misuse clinic seeking help for withdrawal symptoms. Select the most likely drug responsible for each of the withdrawal symptoms described below

Nausea, diffuse aches & pains, goose flesh, runny nose/eyes	Choose... ▾
Profound lethargy, decreased consciousness	Choose... ▾
Restlessness, irritability, insomnia and anxiety	Choose... ▾
Seizures, psychosis, paresthesia	Choose... ▾
Extreme fatigue, hunger, depression	Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Heroin withdrawal- Nausea, general aches & pains, goose flesh, runny nose/eyes. It peaks after 36-72 hours.
- 2) Cocaine withdrawal: Profound lethargy, decreased consciousness. The characteristic feature is intense cravings for cocaine with a striking lack of much physical withdrawal symptoms. The duration of withdrawal lasts from 12-18 hours.
- 3) Cannabis withdrawal- Restlessness, irritability, insomnia and anxiety.
- 4) Benzodiazepines withdrawal- Seizures, psychosis, paresthesia.
- 5) Amphetamine withdrawal- Extreme fatigue, hunger, depression.

(Ref: Maudsley prescribing guidelines- Pg 329)

The correct answer is: Nausea, diffuse aches & pains, goose flesh, runny nose/eyes → Heroin withdrawal, Profound lethargy, decreased consciousness → Cocaine withdrawal, Restlessness, irritability, insomnia and anxiety → Cannabis withdrawal, Seizures, psychosis,

paresthesia → Benzodiazepines withdrawal, Extreme fatigue, hunger, depression → Amphetamine withdrawal



Question 4

Not answered

Marked out of 4.00

Substance misuse (1)

For each of the following clinical presentations, which one of the listed options is the most likely diagnosis?

A young woman is brought to the hospital by her friend after having a seizure. On examination her blood pressure was raised with tachycardia and is noted to have congestion of her nasal mucosa. She claims to be a Hollywood starlet and says that you are preventing her from marrying someone famous. Her friend explains that she works as a prostitute to pay for her drugs

Choose... 

An 18-year-old gentleman who gives a history of panic attacks, anxiety, and vivid flashbacks. He has recently become more withdrawn, acted inappropriately and showed cognitive difficulties. He has a conjunctival injection and a cough

Choose... 

A young man with a history of polysubstance abuse is sweating profusely; he appears agitated and has tachycardia. He describes features of depersonalisation and claims to "see the colour red" while listening to the New York Philharmonic.

Choose... 

A 23-year-old woman was brought to the A& E department, and she has a history of substance misuse. She has bradycardia, hypotension, subnormal body temperature and pinpoint pupils.

Choose... 

Your answer is incorrect.

Explanation:

1) Nasal congestion is suggestive of cocaine intoxication. Clinical features include increased energy, increased confidence, heightened self-esteem, euphoria, and diminished need for sleep. The Physical effects of cocaine include nasal perforation on snorting, cerebral infarctions, subarachnoid, intraparenchymal, and intraventricular hemorrhages, TIAs, Seizures (3 to 8% of A&E visits), myocardial infarctions and arrhythmias.

2) Cannabis intoxication is characterized by impaired motor coordination, euphoria, and sensation of slowed time, social withdrawal, conjunctival injection, cough, increased appetite (munchies), dry mouth and tachycardia. Impairment in motor skills remains after the euphoriant effects are gone.

3) This patient is experiencing synaesthesia, which is a feature of LSD intoxication. Clinical features include marked anxiety or depression, ideas of reference, fear of losing one's mind, paranoid ideation, subjective intensification of perceptions, depersonalization, derealization, illusions, hallucinations, synesthesias in a state of full alertness. Physical features include sweating, tachycardia, pupillary dilation, tachycardia, sweating, palpitations, blurring of vision, tremors and incoordination.

4) Opiate intoxication: Clinical features include initial euphoria, slurred speech, impairment in attention or memory, Apathy & dysphoria follows, psychomotor agitation or retardation, pupillary constriction (if pupillary dilation is seen the overdose may be very severe and anoxia has set in), drowsiness or coma.

The correct answer is: A young woman is brought to the hospital by her friend after having a seizure. On examination her blood pressure was raised with tachycardia and is noted to have congestion of her nasal mucosa. She claims to be a Hollywood starlet and says that you are preventing her from marrying someone famous. Her friend explains that she works as a prostitute to pay for her drugs → Cocaine intoxication, An 18-year-old gentleman who gives a history of panic attacks, anxiety, and vivid flashbacks. He has recently become more withdrawn, acted inappropriately and showed cognitive difficulties. He has a conjunctival injection and a cough → Cannabis intoxication, A young man with a history of polysubstance abuse is sweating profusely; he appears agitated and has tachycardia. He describes features of depersonalisation and claims to "see the colour red" while listening to the New York Philharmonic. → LSD intoxication, A 23-year-old woman was brought to the A& E department, and she has a history of substance misuse. She has bradycardia, hypotension, subnormal body temperature and pinpoint pupils. → Opiate intoxication



Question 5

Not answered

Marked out of 3.00

Substance misuse (2)

For each of the following clinical presentations, which one of the listed options is the most likely diagnosis?

A 25-year-old lady who is on a controlled drug prescription went on a short holiday for three days and forgot to take her prescribed medication. When she returned home, she developed diarrhoea, running nose and goose flesh. On examination, she had dilated pupils and tachycardia.

Choose...

A 30-year-old gentleman presented to the A&E with paranoid delusions and prominent visual hallucinations of very rapid onset following ingesting of a street drug. On examination, he had tachycardia, hypertension and pupillary dilatation.

Choose...

A 27-year-old woman presented to the A&E in an agitated state. She has not had her anti-anxiety medication for last three days. She complains of being anxious and has insomnia. On examination, she has tachycardia and tremor.

Choose...

Your answer is incorrect.

Explanation:

1) Opiate withdrawal effects: Morphine and heroin withdrawal syndrome begins 6 to 8 hours after the last dose, peaks in 2 days and reduces in a week usually. The clinical features include dysphoric mood, nausea or vomiting, muscle aches, lacrimation or rhinorrhea, pupillary dilation, piloerection (goose flesh), or sweating, diarrhea, yawning, fever and insomnia.

2) Amphetamine Intoxication effects: Rapid onset of psychotic symptoms is suggestive of amphetamine intoxication. Clinical features include tachycardia or bradycardia - arrhythmias may be fatal, pupillary dilation, raised or lowered blood pressure, excessive sweating or chills, nausea or vomiting, psychomotor agitation or retardation, muscular weakness, respiratory depression, chest pain, or cardiac arrhythmias. Psychological effects such as euphoria, changes in sociability, anxiety, tension, stereotyped behaviours, impaired judgment, etc. The hallmark of amphetamine-induced psychosis is the paranoia. Sensitisation is suggested to be the biological mechanism behind stimulant induced psychosis.

3) Benzodiazepine withdrawal: Features include autonomic hyperactivity, increased tremor, insomnia, nausea or vomiting, transient visual, tactile, or auditory hallucinations or illusions, psychomotor agitation, anxiety (most prominent), grand mal seizures and kinaesthetic hallucinations reported in a few.

The correct answer is: A 25-year-old lady who is on a controlled drug prescription went on a short holiday for three days and forgot to take her prescribed medication. When she returned home, she developed diarrhoea, running nose and goose flesh. On examination, she had dilated pupils and tachycardia. → Opiate withdrawal, A 30-year-old gentleman presented to the A&E with paranoid delusions and prominent visual hallucinations of very rapid onset following ingesting of a street drug. On examination, he had tachycardia, hypertension and pupillary dilatation. → Amphetamine intoxication, A 27-year-old woman presented to the A&E in an agitated state. She has not had her anti-anxiety medication for last three days. She complains of being anxious and has insomnia. On examination, she has tachycardia and tremor. → Benzodiazepine withdrawal



Question 6

Not answered

Marked out of 3.00

States related to substance use

Choose one diagnostic option for each question below:

A 22-year-old university student develops intense pounding headaches, marked fatigue anxiety and nausea immediately after his exams. He denies using alcohol or smoking.

Choose... ▾

A 30-year-old man has panic attacks and a recurrent cough. He has a red eye on examination but has no needle marks. He appears quite apathetic and unconcerned.

Choose... ▾

A 44-year-old truck driver presents with intense paranoia and anxiety; he has tachycardia and reports feeling 'ants under his pants'.

Choose... ▾

Your answer is incorrect.

Explanation:

1) The diagnosis is caffeine withdrawal. It is not recognized in DSM. But research criteria indicate following prominent features such as headache, marked fatigue or drowsiness, marked anxiety or depression, nausea or vomiting. It can be seen even at low doses from 100mg/day, beginning within 12 hours, peaking at 24-48 hours and lasting up to 1 week.

2) The diagnosis is cannabis intoxication. The red eye on examination is due to conjunctival injection, which is a feature of cannabis intoxication. Other features include euphoria, impaired coordination, the sensation of slowed time, increased appetite, dry mouth, recurrent cough and tachycardia.

3) Cocaine intoxication: Nasal congestion is suggestive of cocaine intoxication. Clinical features include increased energy, increased confidence, heightened self-esteem, euphoria, and diminished need for sleep. With high doses agitation, intense anxiety, irritability, impaired judgment, impulsive aggression, some symptoms of mania and paranoia are noted.

The correct answer is: A 22-year-old university student develops intense pounding headaches, marked fatigue anxiety and nausea immediately after his exams. He denies using alcohol or smoking. → Caffeine withdrawal, A 30-year-old man has panic attacks and a recurrent cough. He has a red eye on examination but has no needle marks. He appears quite apathetic and unconcerned. → Cannabis

intoxication., A 44-year-old truck driver presents with intense paranoia and anxiety; he has tachycardia and reports feeling 'ants under his pants'. → Cocaine intoxication



Question 7

Not answered

Marked out of 5.00

Patterns of drug use

For each of the following descriptions, identify the patterns of use of illegal drugs

Drug use limited to certain situations like parties

Choose...



Use of drugs to explore their effects

Choose...



Continued use may be motivated more by the desire to avoid withdrawals than by positive drug effects

Choose...



Regular but non-dependent use

Choose...



Non-dependent use of variety of drugs

Choose...



Your answer is incorrect.

Explanation: A number of patterns of use of illegal drugs can be recognized.

1) Situational use- Drug use limited to certain situations like parties, raves, etc. Mainly drugs with stimulant/hallucinogenic properties.

2) Experimental use- Use of drugs to explore effects.

3) Dependent use- Use of a drug for which a dependence syndrome has developed. Continued use may be motivated more by the desire to avoid withdrawals than by positive drug effects, which may have diminished due to the development of tolerance.

4) Recreational use- Regular but non-dependent use. It may progress to dependent use.

5) Polydrug use-Non-dependent use of variety of drugs

The correct answer is: Drug use limited to certain situations like parties → Situational use, Use of drugs to explore their effects → Experimental use, Continued use may be motivated more by the desire to avoid withdrawals than by positive drug effects → Dependent use, Regular but non-dependent use → Recreational use, Non-dependent use of variety of drugs → Polydrug use

Question 8

Not answered

Marked out of 2.00

Alcohol-related issues (2)

Choose one option for each question below

A chronic alcoholic has stopped drinking alcohol for last three days. He has been sleeping poorly, extremely anxious, agitated, shaky and is trying to catch imaginary ants all across the room

Choose...

A chronic alcoholic was admitted to psychiatric ward five days ago with a history of confusion. On examination, he had severe anterograde amnesia and was confabulating.

Choose...

Your answer is incorrect.

Explanation:

1) Case 1 is Delirium Tremens: In around 5% patients suffering from alcohol withdrawal symptoms, delirium tremens may set in. In this condition, disturbed autonomic functions (pulse, temperature and blood pressure changes in either direction), clouded consciousness with hallucinations (often Lilliputian) and agitation can occur.

2) Case 2 is Korsakoff's psychosis. About 80 percent of alcoholic patients recovering from Wernicke's encephalopathy develop Korsakoff's amnesic syndrome. It is characterized by marked deficits in anterograde and to some extent retrograde episodic memory, apathy, an intact sensorium, and relative preservation of other intellectual abilities such as attention, procedural memory and working memory.

(Ref: Retrieved from <http://www.enetmd.com/content/alcohol-and-psychiatric-and-physical-disorders>)

The correct answer is: A chronic alcoholic has stopped drinking alcohol for last three days. He has been sleeping poorly, extremely anxious, agitated, shaky and is trying to catch imaginary ants all across the room → Delirium tremens, A chronic alcoholic was admitted to psychiatric ward five days ago with a history of confusion. On examination, he had severe anterograde amnesia and was confabulating. → Korsakoff's psychosis.

Question 9

Not answered

Marked out of 5.00

Alcohol history

Choose one option for each clinical situation given below

A couple present to A&E and both smell of alcohol. The woman reports that her boyfriend had a fall after a quarrel and bleeding from his ears. He had worsening headache and vomiting. On examination, he has upgoing plantar, brisk reflexes and hemiparesis. He appears confused, with unilateral plantar extensor and mild weakness of the left arm.

Choose... ▾

A 44-year-old man has been brought by his wife to the psychiatric clinic as he often binge drinks and forgets where he was or what he did during the episodes of drinking.

Choose... ▾

A chronic alcoholic is prescribed an antibiotic to treat suspected amoebiasis. He develops intense flushing and hypotension.

Choose... ▾

A 44-year-old gentleman stopped drinking three months ago. However, he has started hearing voices, which doesn't make any sense and are threatening to him

Choose... ▾

A 33-year-old gentleman was arrested by the police following his disturbed behaviour after consuming an only small amount of alcohol reportedly. He was agitated and confused. He 'cleared up' after sleeping for 8 hours in the prison cell.

Choose... ▾

Your answer is incorrect.

Explanation:

Case 1- The patient has suffered a head injury, with possible extradural haemorrhage that can cause severe headaches, vomiting and neurological signs.

Case 2 is alcoholic blackouts. Blackouts are discrete episodes of anterograde amnesia that occur in association with alcohol intoxication. During a blackout (at the time of intoxication), remote memory is intact but patients experience specific short-term episodic memory deficit (they can talk about their childhood, etc., but cannot remember what topic the conversation is about or how did they come to where they were).

Case 3- Disulfiram reaction. Drugs like metronidazole, used for the treatment of amoebiasis, could cause this, leading to flushing, nausea, vomiting, headache, tachycardia and hypotension. Metronidazole is considered as a deterrent agent, also called alcohol sensitizing drugs.

Case 4 is Alcoholic hallucinosis. The most common hallucinations are unstructured sounds or voices that may be characteristically malign and threatening. The hallucinations usually last less than a week, when patients believe in the hallucinations though afterwards they may realise the untrue nature.

Case 5 is Pathological intoxication, also called idiosyncratic alcohol intoxication. It is a severe behavioural reaction developing rapidly after consumption of a small amount of alcohol. Confusion, hallucinations, high psychomotor agitation, impulsive and aggressive behaviour with risk to self or others is noted; this may last for few hours and terminate in prolonged sleep.

The correct answer is: A couple present to A&E and both smell of alcohol. The woman reports that her boyfriend had a fall after a quarrel and bleeding from his ears. He had worsening headache and vomiting. On examination, he has upgoing plantar, brisk reflexes and hemiparesis. He appears confused, with unilateral plantar extensor and mild weakness of the left arm. → Extradural haemorrhage, A 44-year-old man has been brought by his wife to the psychiatric clinic as he often binge drinks and forgets where he was or what he did during the episodes of drinking. → Alcoholic memory blackouts, A chronic alcoholic is prescribed an antibiotic to treat suspected amoebiasis. He develops intense flushing and hypotension. → Disulfiram reaction, A 44-year-old gentleman stopped drinking three months ago. However, he has started hearing voices, which doesn't make any sense and are threatening to him → Alcoholic hallucinosis, A 33-year-old gentleman was arrested by the police following his disturbed behaviour after consuming an only small amount of alcohol reportedly. He was agitated and confused. He 'cleared up' after sleeping for 8 hours in the prison cell. → Pathological intoxication.



Question 10

Not answered

Marked out of 5.00

Alcohol Abuse & Investigations

For the statements given below, identify the most appropriate test for patients with alcohol abuse from the list provided:

This is a sensitive marker of alcohol abuse

Choose...

This test accurately predicts recent alcohol use and can be useful in detecting relapse

Choose...

This test is more specific but less useful in detecting relapse

Choose...

This is the best screening tool in primary care

Choose...

This forms part of the routine assessment of patients in follow-up under a supervised detoxification programme

Choose...

Your answer is incorrect.

Explanation:

- 1) Gamma-glutamyl transpeptidase (GGT) is a sensitive marker of alcohol abuse. Levels return to normal 2-4 weeks after abstinence and, therefore, helpful in identifying those who have relapsed after treatment.
- 2) Carbohydrate deficient transferrin (CDT) accurately predicts recent alcohol abuse and can be useful in detecting relapse. Carbohydrate-deficient transferrin is considered to be the single most specific and sensitive test for detecting heavy alcohol use over last ten days. But due to a high degree of intersubject variability it is best to compare CDT levels with patient's baseline.
- 3) Mean corpuscular volume (MCV): Elevated MCV is more specific but less sensitive than GGT for alcohol abuse. Due to the long life span of RBCs (120 days), MCVs may not return to normal for 2-4 months after abstinence and so is less useful in detecting relapse.
- 4) In primary care, AUDIT is the best screening tool (Alcohol Use Disorders Identification Test). It is a 10-item questionnaire, covering quantity, frequency, inability to control drinking, withdrawal relief, loss of memory, injury and concern by others. A score of 8 or more suggest that the person is drinking to a degree that is harmful or hazardous, whereas a score of 13 or more in women and 15 or more in

men is indicative of dependent drinking. It is a very useful and widely used scale, recommended by WHO for primary care use.

5) A breathalyser allows an objective measurement of the breath alcohol and forms part of the routine assessment of patients presenting with alcohol problems or in follow-up under supervised detoxification programme.

The correct answer is: This is a sensitive marker of alcohol abuse → Gamma glutamyl transpeptidase, This test accurately predicts recent alcohol use and can be useful in detecting relapse → Carbohydrate deficient transferrin, This test is more specific but less useful in detecting relapse → Mean corpuscular volume, This is the best screening tool in primary care → AUDIT, This forms part of the routine assessment of patients in follow-up under a supervised detoxification programme → Breathalyser



Question 11

Not answered

Marked out of 5.00

Alcohol screening tools (1)

Choose the best option for each of the statements given below

This is the most widely used screening tool for alcoholism but it does not ask about the frequency of alcohol use

Choose...

A brief structured interview for general practice use

Choose...

This tool was developed for detecting dependent drinkers, and the primary focus is on lifetime rather than current occurrence of alcohol related problems

Choose...

Shorter version of AUDIT used in busy A&E and hospital environments

Choose...

This is a measure of recent alcohol consumption and an objective measure of alcohol intoxication in A&E departments

Choose...

Your answer is incorrect.

Explanation:

1) CAGE is the most widely used screening tool. It is a brief screening questionnaire for the identification of at-risk drinking. But CAGE does not ask about the frequency of alcohol consumption, levels of consumption or episodes of heavy drinking, all factors that can identify patients in the earlier stages of problem drinking.

2) AUDIT also known as Alcohol use disorders identification test is designed to be used as a brief structured interview or self-report questionnaire. It is suitable for general practice use. It is a 10-item questionnaire, covering quantity, frequency, inability to control drinking, withdrawal relief, loss of memory, injury and concern by others. A score of 8 or more indicates that the person is drinking to a degree that is harmful or hazardous, whereas a score of 13 or more in women and 15 or more in men is indicative of dependent drinking. It is a very useful and widely used scale, recommended by WHO for primary care use.

3) Michigan Alcohol Screening Test MAST was developed for detecting dependent drinkers. It has 25 questions that relate to respondents self-appraisal of problems frequently associated with excessive drinking. The major drawback of MAST is that the focus is on lifetime rather than the current occurrence of alcohol-related problems. Hence, one may miss problem drinking at early stages (Nilssen & Cone, 1994).

4) The Fast alcohol screening test (FAST) was developed from AUDIT as a shorter version to use in busy A&E and hospital environments. 5) Blood alcohol concentration is a measure of recent alcohol consumption and an objective measure of alcohol intoxication in A&E departments. It is measured in mg per 100 ml blood (mg %). It correlates well with breath alcohol measured by the breathalyzer.

(Ref: Retrieved from http://www.alcoholism.org/teste/testcage_en.html)

The correct answer is: This is the most widely used screening tool for alcoholism but it does not ask about the frequency of alcohol use → CAGE questionnaire, A brief structured interview for general practice use → Alcohol use disorders identification test (AUDIT), This tool was developed for detecting dependent drinkers, and the primary focus is on lifetime rather than current occurrence of alcohol related problems → Michigan alcohol screening test (MAST), Shorter version of AUDIT used in busy A&E and hospital environments → Fast alcohol screening test (FAST), This is a measure of recent alcohol consumption and an objective measure of alcohol intoxication in A&E departments → Blood alcohol concentration



Question 12

Not answered

Marked out of 4.00

Pharmacology of Substance Use

Choose the drug used in treating addiction related issues that best fits the given description

Aldehyde dehydrogenase inhibitor

Choose... ▾

Non-selective long-acting opiate antagonist

Choose... ▾

Mu opioid receptor partial agonist

Choose... ▾

Shorter acting mu opioid antagonist

Choose... ▾

Your answer is incorrect.

Explanation:

1) Disulfiram: Inhibits aldehyde dehydrogenase. It leads to accumulation of acetaldehyde if alcohol is consumed producing unpleasant reactions. It is used as a prophylactic adjunctive pharmacotherapy to prevent alcohol intake in alcohol dependence.

2) Naltrexone is a longer acting opioid mu antagonist whereas naloxone is a shorter acting opioid mu antagonist. Naltrexone is used to help prevent relapse in formerly opioid-dependent patients who are detoxified and have remained opioid-free for at least a week.

3) Buprenorphine is a partial opioid agonist. At lower doses it has mild agonistic effects; at higher doses it has antagonistic effects. It is used sublingually as an adjunct in managing opioid dependence.

4) Naloxone is a shorter acting opioid mu antagonist.

The correct answer is: Aldehyde dehydrogenase inhibitor → Disulfiram, Non-selective long-acting opiate antagonist → Naltrexone, Mu opioid receptor partial agonist → Buprenorphine, Shorter acting mu opioid antagonist → Naloxone

Question 13

Not answered

Marked out of 5.00

Stages of change I

Identify the most appropriate stage of behaviour change as per Prochaska and DiClemente's model

Mr. T has not made a change to his drinking behaviour. However, he has started appreciating the costs and benefits of their behaviour

Choose... ▾

It is the final stage in the cycle in patients who are successful in negotiating the previous stages

Choose... ▾

Mr. P is content with his behaviour and is unlikely to express any need for change. He is not recognising that the substance was contributing to his problems

Choose... ▾

Mr. Q has been attempting to change his problematic smoking habits for the last four weeks.

Choose... ▾

Mr. Z plans behavioural change by setting clear goals. He also assesses past achievements and failures in his life.

Choose... ▾

Your answer is incorrect.

Explanation:

- 1) In the contemplation stage, the user may accept that there is a problem and begins to look at both the positive and negative aspects of continued drug use. The person has become aware of why the behaviour is a problem but is ambivalent about changing. In maintenance, the person has been able to sustain the change and avoid reverting to problem behaviour for a significant period.
- 2) In successful patients, this usually is the last stage that continues lifelong. It is a stage of maintaining gains made and attempting to improve those areas of life harmed by drug use.
- 3) In the pre-contemplation stage a person is not even considering changing his or her behaviour, does not see the behaviour as a problem, minimizes and denies associated risks, and avoids information to the contrary.
- 4) In action, the person has implemented a plan and is changing the behaviour. It is the point of motivation, where the user attempts change. A variety of routes exists by which change may be attempted, which may or may not include medical services.
- 5) In the ready for action stage, the person is preparing to make plans for change. It also involves planning, preparation and self-evaluation.

The correct answer is: Mr. T has not made a change to his drinking behaviour. However, he has started appreciating the costs and benefits of their behaviour → Contemplation, It is the final stage in the cycle in patients who are successful in negotiating the previous stages → Maintenance, Mr. P is content with his behaviour and is unlikely to express any need for change. He is not recognising that the substance was contributing to his problems → Pre-contemplation, Mr. Q has been attempting to change his problematic smoking habits for the last four weeks. → Action, Mr. Z plans behavioural change by setting clear goals. He also assesses past achievements and failures in his life. → Ready for action



Question 14

Not answered

Marked out of 5.00

Receptor Systems

Identify the drug associated with each of the receptor systems given below:

Opioid receptor system	Choose... ▾
GABA receptor system	Choose... ▾
Acetylcholine receptor system	Choose... ▾
Glutamate receptor system	Choose... ▾
Adenosine receptors system	Choose... ▾

Your answer is incorrect.

Explanation:

- 1) Opiates such as heroin, morphine, codeine, etc bind to specific sites in the brain labeled opioid receptors.
- 2) Ethanol (alcohol) acts on the GABA receptor system. It has effects on dopaminergic neurons in the ventral tegmental area and noradrenergic neurons in the locus coeruleus. Other drugs, which have an effect on GABA receptor system, include benzodiazepines and barbiturates.
- 3) Nicotine has effects on the central nervous system through nicotinic receptors, which is a subclass of acetylcholine receptors.
- 4) Phencyclidine binds to specific receptor sites for glutamate, which is an excitatory amino acid.
- 5) Caffeine acts as an antagonist of the adenosine receptors, and so increases the intraneuronal cAMP. Caffeine at high doses can affect dopamine and noradrenergic neurons.

The correct answer is: Opioid receptor system → Heroin, GABA receptor system → Ethanol, Acetylcholine receptor system → Nicotine, Glutamate receptor system → Phencyclidine, Adenosine receptors system → Caffeine



Question 15

Not answered

Marked out of 5.00

Management of substance use disorders

For each of the following clinical situations, identify the most appropriate treatment option:

Mr. X is a 27-year-old gentleman who presents with paranoia and persecutory ideas. He is drinking to self-medicate. He has some motivation to stop drinking

Miss. Y is a 32-year old woman who comes in with her boyfriend. She has a long-standing history of alcohol dependence and is unsure if she wants to stop

Mr. Z is a 35-year-old gentleman who has been abstinent for three months and wants to continue remaining abstinent

Mr. P is a 55-year-old man with alcohol dependence who has had an inpatient detoxification and is motivated to stay dry. He has a history of ischemic heart disease, diabetes and hypertension. He is keen to have a medication as an adjunct to counselling.

Mr. Q is taking medication for sleeplessness prescribed by his GP. He has read that this drug has a dangerous interaction with alcohol. Identify the drug.

Your answer is incorrect.

Explanation:

- 1) This patient has psychotic symptoms and should be treated with antipsychotic medication.
- 2) Motivational interviewing is a technique aimed at enabling a patient to move through the stages of change to the point where action can be contemplated. The interviewer aims to aid the patient in explaining why they should change their behaviour and how this will be achieved.
- 3) Disulfiram can act as an adjunct to alcohol detoxification therapy and allow the patient's relatives/employers regain confidence in their ability to remain abstinent.

4) Acamprosate could be tried for patients who wish to remain abstinent from alcohol. Disulfiram is contraindicated in patients with hypertension and coronary artery disease.

5) Chlormethiazole can cause respiratory depression and, therefore, should not be used.

The correct answer is: Mr. X is a 27-year-old gentleman who presents with paranoia and persecutory ideas. He is drinking to self-medicate. He has some motivation to stop drinking → Olanzapine, Miss. Y is a 32-year old woman who comes in with her boyfriend. She has a long-standing history of alcohol dependence and is unsure if she wants to stop → Motivational interviewing, Mr. Z is a 35-year-old gentleman who has been abstinent for three months and wants to continue remaining abstinent → Disulfiram, Mr. P is a 55-year-old man with alcohol dependence who has had an inpatient detoxification and is motivated to stay dry. He has a history of ischemic heart disease, diabetes and hypertension. He is keen to have a medication as an adjunct to counselling. → Acamprosate, Mr. Q is taking medication for sleeplessness prescribed by his GP. He has read that this drug has a dangerous interaction with alcohol. Identify the drug. → Chlormethiazole



Question 16

Not answered

Marked out of 4.00

Neonatal drug effects

Given the clinical picture below, identify the most appropriate diagnoses from list of options attached

Newborn baby with low birth weight, small head circumference and dysmorphic facial features

Choose...



Newborn baby who is very irritable, high-pitched cry, cries a lot with shakiness, poor feeding, Sleep-wake abnormalities and vomiting

Choose...



Newborn shows mild tremors, intermittent irritability and increased need for handling. Neonatal reflexes are normal.

Choose...



Newborn baby with low birth weight, breathing difficulties, hypotonic muscles and irritability

Choose...



Your answer is incorrect.

Explanation:

- 1) Foetal alcohol syndrome is the commonest non-genetic cause of learning disability. Intellectual impairment, dysmorphic facial features in the form of low birth weight, small head circumference and disruptive behaviour is noted.
- 2) Opiate withdrawal: It usually begins 24-48 hours after birth, depending on the time of last dose. The essential features are hyperirritability, gastrointestinal dysfunction, respiratory distress, and vague autonomic symptoms (e.g., yawning, sneezing, mottling, fever). Tremors and jittery movements, high-pitched cries, increased muscle tone, and irritability are common. Normal reflexes may be exaggerated. Loose stools are common, leading to possible electrolyte imbalances and diaper dermatitis.
- 3) Nicotine withdrawal: Mild signs such as fine tremors and variations in tone; recent data indicates subtle neonatal behaviours, such as poor self-regulation and an increased need for handling.
- 4) Floppy baby syndrome: Babies exposed to benzodiazepines during pregnancy are at risk for the following- breathing difficulties, low birth weight, floppy muscles, unstable body temperature, alterations in heart rate/function, irritability, convulsions, etc. Prenatal exposure to Benzodiazepine can cause toxicity and withdrawal effects at birth. Flumazenil, a Benzodiazepine receptor antagonist, can reverse many

effects of BDZs and has been used in emergencies in some BDZ-exposed neonates, successfully reversing most of the above adverse effects. (Ref: Retrieved from <http://emedicine.medscape.com/article/978492-clinical>)

The correct answer is: Newborn baby with low birth weight, small head circumference and dysmorphic facial features → Fetal Alcohol Syndrome, Newborn baby who is very irritable, high-pitched cry, cries a lot with shakiness, poor feeding, Sleep-wake abnormalities and vomiting → Opiate withdrawal, Newborn shows mild tremors, intermittent irritability and increased need for handling. Neonatal reflexes are normal. → Nicotine withdrawal, Newborn baby with low birth weight, breathing difficulties, hypotonic muscles and irritability → Floppy baby syndrome



Question 17

Not answered

Marked out of 5.00

Street names of drugs (1)

Identify the conventional drug name for each of the street drugs

Brown	Choose...
Jellies	Choose...
Speed	Choose...
Mushies	Choose...
Ketamine	Choose...

Your answer is incorrect.

Explanation: Heroin has various street names such as Brown, Gear, Skag, and smack. The street name for temazepam is Jellies and for diazepam is vallies (derived from Valium, the trade name). Amphetamines have different street names such as Billy, Speed, sulph, and whizz. Psilocybin mushrooms are called as mushies. The street names of ketamine include 'special K' and 'vitamin K'.

The correct answer is: Brown → Heroin, Jellies → Temazepam, Speed → Amphetamine, Mushies → Psilocybin mushrooms, Ketamine → Special K

Question 18

Not answered

Marked out of 3.00

Alcohol screening tools (2)

Choose the best tool to be used for each of the clinical situations given below

A young man went to his GP for consultation. He is drinking alcohol excessively and has problems with his sleep. He has marital problems. Which scale is useful to assess the severity of drinking?

Choose...

A young man came to the A&E with slurred speech and confusion. Which test do you use?

Choose...

A 40-year-old patient with a diagnosis of schizophrenia is not happy with his new medication. He feels there has been no change in his mental state.

Choose...

Your answer is incorrect.

Explanation:

1) Alcohol use disorders identification test (AUDIT) is designed to be used as a brief structured interview or self-report questionnaire. It is suitable for general practice use. It is a very useful and widely used scale, recommended by WHO for primary care use.

2) When a patient presents in emergency situations, long alcohol screening instruments must not be employed. CAGE, though short, is also not the priority as the patient is currently in a state of confusion. It is important to assess and quantify the degree of confusion using either Glasgow coma scale (preferably) or Abbreviated mental test score.

3) PANSS scale is sensitive to change after intervention and can be used to quantify and demonstrate treatment response. Another option is BPRS, if provided.

The correct answer is: A young man went to his GP for consultation. He is drinking alcohol excessively and has problems with his sleep. He has marital problems. Which scale is useful to assess the severity of drinking? → Alcohol use disorders identification test (AUDIT), A young man came to the A&E with slurred speech and confusion. Which test do you use? → Abbreviated mental test score, A 40-year-old patient with a diagnosis of schizophrenia is not happy with his new medication. He feels there has been no change in his mental state. → PANSS

Question 19

Not answered

Marked out of 2.00

Alcohol & complications

Choose one option for each question below

A chronic alcoholic has stopped drinking alcohol for last three days. He has been sleeping poorly, extremely anxious, agitated, shaky and is trying to catch imaginary ants all across the room

Choose...

A chronic alcoholic was admitted to psychiatric ward five days ago with a history of confusion. On examination, he had severe anterograde amnesia and was confabulating.

Choose...

Your answer is incorrect.

Explanation:

Case 1 refers to Delirium Tremens: In around 5% patients suffering from alcohol withdrawal symptoms, delirium tremens may set in. In this condition, disturbed autonomic functions (pulse, temperature and blood pressure changes in either direction), clouded consciousness with hallucinations (often Lilliputian) and agitation can occur.

Case 2 is likely to be Korsakoff's psychosis. About 80 percent of alcoholic patients recovering from Wernicke's encephalopathy develop Korsakoff's amnesic syndrome. It is characterized by marked deficits in anterograde and to some extent retrograde episodic memory. Other features include apathy, an intact sensorium, and relative preservation of other intellectual abilities such as attention, procedural memory, and working memory.

(Ref: Retrieved from <http://www.enetmd.com/content/alcohol-and-psychiatric-and-physical-disorders>)

The correct answer is: A chronic alcoholic has stopped drinking alcohol for last three days. He has been sleeping poorly, extremely anxious, agitated, shaky and is trying to catch imaginary ants all across the room → Delirium tremens, A chronic alcoholic was admitted to psychiatric ward five days ago with a history of confusion. On examination, he had severe anterograde amnesia and was confabulating. → Korsakoff's psychosis.

Question 20

Not answered

Marked out of 5.00

Drug detection in urine

How long could the following drugs be detected in urine?

Amphetamines	Choose... ▾
Alcohol	Choose... ▾
Benzodiazepines	Choose... ▾
Methadone	Choose... ▾
LSD	Choose... ▾

Your answer is incorrect.

Explanation:

Amphetamines could be detected in the urine for up to 48 hours and alcohol for up to 12 hours. **Benzodiazepines** could be detected in the urine for up to 3 days (depending on $t_{1/2}$). The half-life of **methadone** is longer than heroin and could be detected for three days or more in urine. **Lysergic acid diethylamide (LSD)** could be detected in the urine for less than 24 hours in average users.

The correct answer is: Amphetamines → Up to 48 hours, Alcohol → Up to 12 hours, Benzodiazepines → 1-3 days, Methadone → 3 days or more, LSD → Up to 24 hrs

Question 21

Not answered

Marked out of 5.00

Street names of drugs (2)

Identify the street name for each of the conventional names of drugs given below

Lysergic acid diethylamide (LSD)

Choose... ▾

Phencyclidine (PCP)

Choose... ▾

Cannabis

Choose... ▾

Cocaine

Choose... ▾

3,4 -methylene-dioxymethamphetamine (MDMA)

Choose... ▾

Your answer is incorrect.

Explanation:

Cannabis has various street names such as blow, dope, grass, hash, marijuana, and skunk (a potent form of cannabis).

Cocaine has various street names such as C, Charlie, coke, snow and crack (free base cocaine).

MDMA has different street names such as Adam, E, Ecstasy, Eccies.

The correct answer is: Lysergic acid diethylamide (LSD) → Acid, Phencyclidine (PCP) → Angel dust, Cannabis → Dope, Cocaine → Charlie, 3,4 -methylene-dioxymethamphetamine (MDMA) → Ecstasy

Question 22

Not answered

Marked out of 5.00

Withdrawal & Intoxication syndromes

For each of the clinical situations described below, choose options from the given list.

A 23-year-old man with vivid unpleasant dreams and increased appetite

Choose... ▾

A 27-year-old woman with pupillary dilatation, yawning and nausea

Choose... ▾

A 19-year-old man with nystagmus, slurred speech, incoordination of movements [Choose Two]

Choose... ▾

A 20-year-old woman with pupillary dilatation, nausea and seizures

Choose... ▾

A 23-year-old man with dysphoric mood, hypersomnia, intense cravings

Choose... ▾

Your answer is incorrect.

Explanation:

The withdrawal effects of amphetamine include vivid, unpleasant dreams, hypersomnia and increased appetite.

Opioid withdrawal features include nausea or vomiting, muscle aches, lacrimation or rhinorrhea, pupillary dilation, piloerection (goose flesh), or sweating, nausea, diarrhea and yawning.

The intoxication effects of inhalants and benzodiazepines are the same and includes dizziness, nystagmus, incoordination, slurred speech, muscle weakness and unsteady gait.

Intoxication effects of amphetamines include tachycardia or bradycardia, pupillary dilation, elevated or lowered blood pressure, perspiration or chills, nausea or vomiting, muscular weakness, respiratory depression, chest pain, or cardiac arrhythmias. The CNS effects include confusion, seizures, dyskinesias, dystonias, or coma.

Cocaine Withdrawal: Effects of cocaine are short-lived - rapid cessation occurs due to rapid metabolism - this can lead to repeated use.

Dysphoria (crash), anhedonia, anxiety, irritability hypersomnolence, and sometimes agitation usually end within 18 hours though in heavy dependent users this can last up to a week, peaking in 3 days. The characteristic feature is an intense craving for cocaine with a striking lack

of physical withdrawal symptoms.

The correct answer is: A 23-year-old man with vivid unpleasant dreams and increased appetite → Amphetamine withdrawal, A 27-year-old woman with pupillary dilatation, yawning and nausea → Opiate withdrawal, A 19-year-old man with nystagmus, slurred speech, incoordination of movements [Choose Two] → Inhalant intoxication, A 20-year-old woman with pupillary dilatation, nausea and seizures → Amphetamine intoxication, A 23-year-old man with dysphoric mood, hypersomnia, intense cravings → Cocaine withdrawal



Question 23

Not answered

Marked out of 4.00

Drug use & presentations

Identify the most likely recreational drug that has been used in each of the following situations:

A 17-year-old man with a known history of substance abuse went on an 'all night party'. Later he presented to A & E with tremors, tachycardia, dilated pupils and sweating

Choose...



A 19-year-old factory worker is trying to build his body for 'the muscle-man show' later this year. He presents to A&E after an episode of aggression at a local pub. He has significant fluid retention and high blood pressure on examination. On further questioning grandiose ideas are evident, and he gives a history of mood swings.

Choose...



A 20-year-old man repeatedly experiences a sensation of rising above his body similar to near-death experiences. He also describes a loss of sense of time and multiple visual distortions during drug trips.

Choose...



A 44-year-old patient has been arrested following a petty theft and after 36 hours in the police cell, he develops seizures.

Choose...



Your answer is incorrect.

Explanation:

Tachycardia or bradycardia, pupillary dilation, elevated or lowered blood pressure, perspiration or chills, nausea or vomiting, psychomotor agitation or retardation, muscular weakness, respiratory depression, chest pain, or cardiac arrhythmias confusion, seizures, dyskinesias, dystonias, or coma, are signs of either cocaine or amphetamine use. (Ref: DSM-IV).

Steroid use can be associated with increased anger, arousal, irritability, hostility, anxiety, somatization, and depression (especially when steroids are not used). 2 to 15 percent of anabolic steroid abusers experience hypomanic or manic episodes. (Ref: Synopsis of Psychiatry - 10th edition pg: 463).

People who take ketamine and phencyclidine (NMDA antagonists) experience speedy feelings, euphoria, bodily warmth, tingling, peaceful floating sensations, and, occasionally, feelings of depersonalization, isolation, and estrangement. (Ref: Synopsis of Psychiatry - 10th Edition: Pg: 453).

The sudden withdrawal of alcohol or benzodiazepine leads to seizures reflects the unmasking of the effects of GABA downregulation and excess glutamatergic activity.

(Retrieved from: Rogawski MA. Update on the Neurobiology of Alcohol Withdrawal Seizures. Epilepsy Curr. 2005 November; 5(6): 225-230 & http://www.healthquality.va.gov/sud/sud_sum_combined.pdf)

The correct answer is: A 17-year-old man with a known history of substance abuse went on an 'all night party'. Later he presented to A & E with tremors, tachycardia, dilated pupils and sweating → Amphetamines, A 19-year-old factory worker is trying to build his body for 'the muscle-man show' later this year. He presents to A&E after an episode of aggression at a local pub. He has significant fluid retention and high blood pressure on examination. On further questioning grandiose ideas are evident, and he gives a history of mood swings. → Anabolic steroids, A 20-year-old man repeatedly experiences a sensation of rising above his body similar to near-death experiences. He also describes a loss of sense of time and multiple visual distortions during drug trips. → Ketamine, A 44-year-old patient has been arrested following a petty theft and after 36 hours in the police cell, he develops seizures. → Alcohol



Question **24**

Not answered

Marked out of 8.00

Categories of substances

Identify the drugs of abuse for each of the categories below

1. Stimulants (choose two answers)
2. Hallucinogens (choose two answers)
3. Depressants (choose two answers)
4. Opiates (choose two answers)

Explanation:

Stimulants- amphetamines, cocaine, MDMA.

Hallucinogens- LSD, Phencyclidine, Mushrooms, Ketamine.

Depressants- alcohol, benzodiazepines, barbiturates, Gammahydroxybutyrate.

Opiates- heroin, methadone, codeine, dihydrocodeine, pethidine, buprenorphine.

Correct Answer is : 1. Cocaine, Amphetamines 2. LSD, Phencyclidine 3. Alcohol, Benzodiazepines 4. Heroin, Pethidine



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